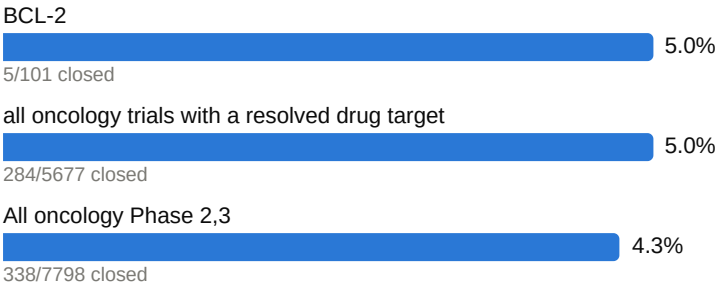


BCL-2: one molecule stopped early across 5 trials

5 stopped trials, all of one molecule, Venetoclax (a small molecule). Against 5.0% for all oncology trials with a resolved drug target and 4.3% across all oncology Phase 2,3 trials in the same window. Rates count trials that stopped early for efficacy, safety or benefit–risk reasons; trials that completed and missed their endpoints are not counted.

5.0% 5 of 101 closed trials (95% CI 2.1%–11.1%)	4 / 0 / 1 efficacy / safety / both (adds to 5)	29.6% of 341 trials have closed · 240 still open or unresolved	0.0% stopped for a biological reason within 3 years of starting (95% CI 0.0%–0.0%; 183 still at risk; 1.5% area-wide)
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How this compares



What is in this class: 183 distinct experimental drugs across 341 trials (small molecule 336, antibody 91, unknown 48, protein 16). Most tested: VENETOCLAX (306), AZACITIDINE (89), OBINUTUZUMAB (38), CYTARABINE (37), IBRUTINIB (31). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 5 stops came from 5 sponsor-asset programmes across 5 sponsors; the largest (Roche) contributed 1; removing that programme's trials from both sides leaves 4.0%. Screened alongside 83 other segments in this area, it does not survive a 10% false-discovery correction ($q=1$, Benjamini-Yekutieli). A further 8 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 12.9%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	1	22	4.5%	0.8%–21.8%
2018–2020	4	52	7.7%	3.0%–18.2%
2021–2024	0	27	0.0%	0.0%–12.5%

Where the stops sit

Efficacy / futility: 4, Efficacy and safety: 1.
Sponsors with most stops: Novartis (1), Roche (1), Ono Pharmaceutical Co., Ltd. (1), M.D. Anderson Cancer Center (1), Fondazione Italiana Linfomi - ETS (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04150029	2	Novartis NVS	SABATOLIMAB, VENETOCLAX, AZACITIDINE	2024-10	Efficacy	Novartis made a decision to early terminate the sabatolimab clinical development program, including the CMBG453C12201 trial, following the negative results from other 2 trials.
NCT04029688	2	Roche	IDASANUTLIN, VENETOCLAX, CYCLOPHOSPHAMIDE ANHYDROUS, TOPOTECAN	2024-05	Efficacy + safety	Study was terminated by the sponsor based on insufficient tolerability and efficacy to proceed beyond Part 1b.
NCT03176277	2	Ono Pharmaceutical Co., Ltd.	TAMNORZATINIB, VENETOCLAX	2022-12	Efficacy	Protocol defined futility criteria
NCT03634228	2	M.D. Anderson Cancer Center	CYTARABINE, MILADEMETAN, VENETOCLAX	2022-04	Efficacy	This study was terminated early due to lack of adequate response, and did not move to the Phase II portion of the study

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT03552692	2	Fondazione Italiana Linfomi - ETS	VENETOCLAX	2020-03	Efficacy	efficacy lower than expected

Method

Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits

Not a failure rate: trials that completed with negative results are not counted, and programs discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 29.6% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

Any mechanism, sponsor or indication, updated weekly. The dataset behind this brief covers every stopped oncology trial with an efficacy or safety signal plus the full denominator universe. Free sample and licensing: clinicaltrialfailures.com/data-licensing

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxCClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.